ETHAMSYLATE

Capillary bleeding
Weak evidence base

CHEMICAL NAME Diethylammonium 2,5-dihydroxybenzenesulfonate (etamsylate)
CLASS Haemostatic agent (capillary stabiliser)
TRADE NAMES Dicynone · Dicynene · Hemsyl · Etamsylate

THERAPEUTIC USES / INDICATIONS

- Menorrhagia (heavy menstrual bleeding).
- Capillary bleeding: post-surgical (ENT, dental, urological), epistaxis.
- Prevention of periventricular haemorrhage in preterm infants (historical use).

PHARMACODYNAMICS

- Increases capillary wall resistance and promotes platelet adhesion (via P-selectin).
- Does not affect coagulation factors or fibrinolysis; no vasoconstriction.

ADMINISTRATION

- **PO** tablets with food or water.
- **IV/IM** injection slowly.
- Can be applied topically to bleeding gums or tooth sockets.

SIDE & ADVERSE EFFECTS

SIDE EFFECTS (COMMON)

- Nausea, headache.
- Skin rash.
- Fever.

ADVERSE EFFECTS (SERIOUS)

- Hypersensitivity reactions, angioedema.
- Transient hypotension (IV).
- Thromboembolism (rare).

DOSAGES

- **Menorrhagia:** 500 mg four times daily from start of bleeding until it stops.
- **Surgical bleeding:** 250–500 mg IV/IM before surgery, repeated every 4–6 h.
- **Children:** 10–15 mg/kg/day divided.

CONTRAINDICATIONS & CAUTIONS

- Porphyria.
- Hypersensitivity (contains sulfite in some injections; asthma).
- **Caution:** history of thrombosis.

KEY INTERACTIONS

- No significant interactions; may be combined with tranexamic acid.

PHARMACOKINETICS

- **Onset:** IV 5–15 min; PO 1 h. **Duration:** 4–6 h.
- Not significantly metabolised.
- Excreted unchanged in urine. **t½:** 2 h.

NURSING MANAGEMENT

- Assess blood loss (pad count, drains, Hb).
- Give IV slowly; monitor BP.
- Watch for allergy, especially in asthmatics.
- Investigate cause of persistent bleeding.

PATIENT EDUCATION

- Menorrhagia: start at the beginning of the period; report if bleeding remains heavy.
- Report rash or swelling.

KEY POINTS

- Capillary stabiliser; ↑ platelet adhesion.
- Used for menorrhagia and capillary oozing.
- No effect on clotting factors.
- Avoid in porphyria.